

Pre-Travel Health Advisory

United States · 7 days · Leisure

Departure: 5 December 2026

Traveller: Dike Wisdom

Low risk

COMPLETED

Apr 24, 2026

TRIP AT A GLANCE	
DURATION	7 days
PURPOSE	Leisure
TRAVELLING	Alone
ACCOMMODATION	Hotel (urban)
INSURANCE	Yes

ITINERARY-SPECIFIC GUIDANCE		
Trip type: ONE_WAY		
This is a one-way journey departing Nairobi, Kenya on 5 December 2026 and arriving in Oklahoma, United States. The traveller is transiting from a medium-risk destination (Kenya) to a low-risk destination (United States). The primary health risks are concentrated in the Kenya departure phase and the transit period. Health preparations must be completed before leaving Nairobi.		
ROUTE GUIDANCE BY STOP		
STOP	COUNTRY	GUIDANCE
Oklahoma	United States	Oklahoma is a low health-risk destination for this traveller. Standard food and water safety applies; tap water is safe. No malaria risk. No tropical disease precautions required beyond completing the malaria prophylaxis course for 7 days after leaving Kenya. Monitor for any symptoms acquired during the Kenya leg, particularly fever (malaria), gastrointestinal illness, or STI symptoms. Attend a post-travel health check within 4 weeks of arrival. Ensure travel and health insurance covers the United States, which has high healthcare costs. Confirm health insurance coverage before arrival, particularly if transitioning from travel insurance to a US-based health plan.

HEALTH RISK OVERVIEW		
CATEGORY	LEVEL	SUMMARY
Food and water safety	MODERATE	Nairobi carries a moderate food and water risk. Tap water is generally not recommended for drinking. Oklahoma presents low food and water risk with treated municipal supplies. Strict food and water hygiene should be observed during the Nairobi leg of the journey.
Vector-borne diseases	MODERATE	Nairobi city itself sits at approximately 1,700 metres elevation and carries a lower malaria risk than rural Kenya; however, risk is not zero and depends on

		activities and any excursions outside the city. Dengue and chikungunya are present in Kenya. Oklahoma presents negligible vector-borne disease risk.
Respiratory infections	MODERATE	Influenza circulates year-round in Kenya and seasonally in the United States. COVID-19 remains a background risk in both countries. Planned volunteering and humanitarian activities increase exposure to respiratory pathogens through close contact with others.
Environmental health (heat, sun, air quality)	LOW	Nairobi has a mild climate year-round due to its elevation. Oklahoma in December is typically cool to cold. Sun protection remains relevant in Nairobi. Air quality in both urban settings is generally acceptable, though Nairobi can experience traffic-related pollution.
Injuries and road traffic safety	MODERATE	Road traffic injury risk in Nairobi is elevated. Driving standards, road conditions, and pedestrian safety differ significantly from Western norms. Oklahoma presents standard road safety risks. Solo travel increases personal vulnerability in unfamiliar environments.
Rabies and animal contact	HIGH	Kenya is rabies-endemic. The traveller has indicated planned farm and animal contact activities, which significantly elevates rabies exposure risk. Pre-exposure rabies prophylaxis is strongly recommended before departure. Oklahoma is not rabies-free but human exposure risk in an urban hotel setting is low.
Blood-borne and sexual health	HIGH	The traveller has indicated anticipated casual sexual relationships, inconsistent barrier protection (sometimes), and a prior history of sexually transmitted infection (STI). Kenya has a significant HIV prevalence. This combination represents a high blood-borne and sexual health risk requiring direct and proactive counselling.
Altitude-related illness	LOW	Nairobi sits at approximately 1,700 metres above sea level, which is below the 2,500 metre threshold for acute mountain sickness risk. Oklahoma is at low elevation. No altitude-related illness risk is anticipated for this itinerary.

FLIGHT & JOURNEY HEALTH

VTE Risk Level: **MEDIUM**

PREVENTION MEASURES

- The longest flight leg is 6 hours, which places this traveller in the medium VTE (venous thromboembolism, or blood clot) risk category.
- Wear graduated compression stockings (15 to 30 mmHg) for the duration of the flight.
- Perform regular in-seat leg exercises: ankle circles, heel raises, and calf pumps every 30 to 60 minutes.
- Walk the aisle for 5 minutes every 1 to 2 hours when the seatbelt sign is off.
- Stay well hydrated throughout the flight; avoid excessive alcohol and caffeine, which promote dehydration.
- Avoid sleeping in positions that compress the legs or restrict circulation for extended periods.
- No LMWH (low molecular weight heparin injection) is indicated at this time given no additional high-risk VTE factors reported, but discuss with your clinician if you have any concerns.

MEDICATION TIMING

No current medications reported. No insulin or time-sensitive medication timing adjustments required. If malaria prophylaxis (atovaquone-proguanil) is prescribed,

VACCINATIONS

VACCINE	STATUS	GUIDANCE
Routine immunizations (e.g. MMR, varicella, dTdap, polio /IPV)	Believed up to date	Recommended: verify and update before travel Confirm documentation of MMR (2-dose series), varicella, dTdap (tetanus-diphtheria-pertussis booster if more than 10 years since last dose, or more than 5 years given planned outdoor and farm activities), and polio/IPV. Obtain written records. Gaps should be closed before adding travel-specific vaccines.
Influenza	Unknown	Recommended Annual influenza vaccine is recommended for all travellers. Influenza circulates year-round in Kenya and seasonally in the United States. Particularly relevant given planned volunteering and close-contact activities. Administer before departure.
COVID-19	Unknown	Recommended Ensure COVID-19 vaccination is up to date including any applicable booster doses. Check current United States entry requirements for COVID-19 documentation prior to travel, as requirements may change.
Hepatitis A	Not documented	Recommended Hepatitis A vaccination is recommended for travel to Kenya. Transmission occurs through contaminated food and water. A single dose provides protection for up to 1 year; a booster at 6 to 12 months provides long-term immunity. Administer at least 2 weeks before departure if possible.
Hepatitis B	Not documented	Strongly recommended Hepatitis B vaccination is strongly recommended given the traveller's planned activities (volunteering, humanitarian work, farm and animal contact) and anticipated sexual risk behaviours with inconsistent protection. An accelerated schedule (doses at day 0, day 7, and day 21 with a booster at 12 months) is available if time before departure is limited. Discuss with your clinician.
Typhoid	Not documented	Recommended for Kenya leg Typhoid vaccination is recommended for travel to Kenya, particularly given urban street food exposure risk and planned activities. Either the oral live attenuated vaccine (Ty21a, 4 capsules over 7 days, not suitable if immunocompromised) or the injectable Vi polysaccharide vaccine (single dose, at least 2 weeks before travel) are appropriate options. Discuss with your clinician.
Yellow fever	Not documented	Recommended and may be required Yellow fever vaccination is recommended for travel to Kenya, as Kenya is a yellow fever risk country. Additionally, proof of yellow fever vaccination may

		be required for entry into the United States if arriving from a yellow fever risk country, depending on current US entry requirements. Administer at least 10 days before departure. A single dose provides lifelong protection for most individuals. Discuss with a certified yellow fever vaccination centre.
Japanese encephalitis	Not applicable	Not indicated Japanese encephalitis is not present in Kenya or the United States. This vaccine is not required or recommended for this itinerary.
Meningococcal	Unknown	Consider for Kenya Kenya falls within the African meningitis belt region. Meningococcal disease (MenACWY) vaccination is worth discussing with your clinician, particularly given planned volunteering and humanitarian activities involving close contact with local populations. Discuss with your clinician before departure.
Rabies pre-exposure	Not documented	Strongly recommended Kenya is rabies-endemic and the traveller has indicated planned farm and animal contact activities. Pre-exposure rabies prophylaxis (PrEP) is strongly recommended. The standard schedule is 3 doses on days 0, 7, and 21 or 28. PrEP does not eliminate the need for post-exposure treatment but simplifies it significantly and is critical when access to post-exposure biologics (rabies immunoglobulin) may be limited. Begin the series as early as possible before departure.
Cholera (oral vaccine)	Not documented	Consider for Kenya Cholera risk in Nairobi is generally low for hotel-based urban travellers. However, given planned volunteering and humanitarian activities, which may involve contact with underserved communities or informal settlements, the oral cholera vaccine (Dukoral or Vaxchora) is worth discussing with your clinician. Discuss with your clinician based on the specific nature of volunteering activities.

MALARIA PREVENTION

Risk Level: MODERATE

RECOMMENDED CHEMOPROPHYLAXIS

Atovaquone-proguanil (Malarone) — preferred first-line option for this traveller

Nairobi city itself is at approximately 1,700 metres elevation and is considered lower risk for malaria than rural Kenya; however, malaria transmission does occur and risk is not zero, particularly if the traveller undertakes any excursions outside the city centre or to lower-altitude areas. Given the planned outdoor activities (hiking, farm contact, volunteering), chemoprophylaxis is recommended. Atovaquone-proguanil is preferred: it is well tolerated, has no neuropsychiatric side effects (relevant given anticipated alcohol use), and is suitable for short trips as it is started 1 to 2 days before travel and stopped 7 days after leaving the malaria-risk area. Doxycycline is an alternative but requires strict sun protection (photosensitising) and is taken daily. Mefloquine is not recommended given the traveller's anticipated alcohol consumption and the neuropsychiatric risk profile. Discuss agent selection with your clinician.

MOSQUITO PROTECTION MEASURES

	▪ Apply DEET-based insect repellent (at least 30 to 50 percent DEET) to all exposed skin, especially during dawn and dusk hours when Anopheles mosquitoes are most active.
	▪ Wear long-sleeved shirts and long trousers in light colours during outdoor activities, particularly in the evening.
	▪ Sleep in air-conditioned rooms where possible or use a permethrin-treated bed net if accommodation is not fully screened.
	▪ Consider treating outdoor clothing and gear with permethrin spray for added protection during hiking and farm activities.
	▪ Avoid standing water and areas of dense vegetation at dusk and dawn.
	▪ If fever develops during travel or within 3 months of return, seek immediate medical attention and inform the clinician of travel to a malaria-risk area.
CONTRAINDICATIONS / ALTERNATIVES	No contraindications identified for atovaquone-proguanil based on current questionnaire responses. No known allergies, no chronic conditions, no current medications reported. Confirm with prescribing clinician before dispensing.

MEDICAL ADVICE & RECOMMENDATIONS	
TOPIC	DETAILS
Pre-travel review and vaccination records	Book a pre-travel health consultation with your primary care physician or a certified travel medicine clinic as soon as possible, ideally at least 4 to 6 weeks before your 5 December 2026 departure. Bring all existing vaccination records. Several vaccines require multiple doses or a minimum lead time before they become effective (rabies pre-exposure requires 3 doses over 21 to 28 days; hepatitis B accelerated schedule requires 3 doses; yellow fever requires 10 days minimum). Starting this process promptly is essential. Confirm yellow fever vaccination certificate requirements for US entry from Kenya before departure.
Food and water hygiene	In Nairobi, drink only bottled, boiled, or treated water. Avoid ice in drinks unless you are certain it was made from safe water. Eat food that is freshly cooked and served hot. Avoid raw salads, unpeeled fruit, and street food from vendors with poor hygiene standards. Wash hands thoroughly with soap and water before eating and after using the toilet. Carry alcohol-based hand sanitiser (at least 60 percent alcohol) for situations where handwashing is not possible. In Oklahoma, standard food and water safety applies and tap water is safe to drink.
Vector bite prevention	Apply DEET-based repellent (30 to 50 percent) to all exposed skin when outdoors in Kenya, particularly during dawn and dusk. Wear long-sleeved, light-coloured clothing during outdoor activities including hiking and farm visits. Use permethrin-treated clothing and gear for extended outdoor exposure. Sleep under a permethrin-treated bed net if your hotel room is not fully air-conditioned and screened. Be aware that mosquitoes transmitting dengue and chikungunya are active during daytime hours, so repellent use should not be limited to evenings only.
Sun, heat, and environmental precautions	Nairobi has a mild climate but UV radiation is elevated at its altitude (approximately 1,700 metres) and equatorial latitude. Apply SPF 30 or higher broad-spectrum sunscreen to exposed skin daily, even on overcast days. Wear a wide-brimmed hat and UV-protective sunglasses during outdoor activities. Stay hydrated, particularly during hiking and outdoor

	volunteering. Oklahoma in December will be cool to cold; pack appropriate warm clothing. Air quality in Nairobi can be affected by traffic pollution; those with respiratory sensitivities should be aware.
Injury and road safety	Road traffic injury is a significant risk in Nairobi. Avoid travelling by road at night where possible. Use reputable, pre-booked transport services rather than unmarked taxis. Always wear a seatbelt. If hiring a vehicle, ensure it is roadworthy and insured. Be cautious as a pedestrian; traffic may not observe pedestrian right of way. During hiking and outdoor activities, use appropriate footwear, stay on marked trails, and inform someone of your planned route and expected return time. As a solo traveller, maintain situational awareness at all times and keep emergency contacts readily accessible.
Sexual health and blood exposure	The traveller has indicated anticipated casual sexual relationships, inconsistent condom use, and a prior history of STI. This represents a significant health risk, particularly in Kenya where HIV prevalence is substantially higher than in Nigeria or the United States. Consistent and correct use of condoms (male or female) with every sexual encounter is strongly recommended and provides meaningful protection against HIV, other STIs, and unintended pregnancy. Pre-exposure prophylaxis (PrEP) for HIV prevention is worth discussing with your clinician before departure, given the anticipated risk profile. Post-exposure prophylaxis (PEP) for HIV is available in Kenya and the United States but must be started within 72 hours of potential exposure; know where to access it at your destination. Avoid tattooing, piercing, or any cosmetic procedures abroad unless you can verify sterile professional facilities. Avoid sharing needles, razors, or any items that may carry blood. Consider pre-travel STI screening and plan for post-travel STI and HIV testing after return.
Jet lag, sleep, and mental wellbeing	The time difference between Nairobi (East Africa Time, UTC+3) and Oklahoma (Central Standard Time, UTC-6) is 9 hours. This is a significant time zone shift and jet lag is likely. On arrival in Oklahoma, expose yourself to natural daylight during the day to help reset your circadian rhythm. Maintain regular sleep and meal times as quickly as possible. Avoid excessive alcohol use to manage jet lag, as alcohol disrupts sleep quality and impairs immune function. As a solo traveller, be mindful of isolation and unfamiliar environments. Maintain regular contact with family or friends back home. If you experience significant mood changes, anxiety, or difficulty coping, seek support early.
Malaria and other chemoprophylaxis (state if not indicated)	Malaria chemoprophylaxis is recommended for the Kenya leg of this journey. Atovaquone-proguanil (Malarone) is the preferred agent for this traveller. Begin 1 to 2 days before entering the malaria-risk area and continue for 7 days after leaving. Take with food or a milky drink to improve absorption and reduce nausea. Do not rely on chemoprophylaxis alone; mosquito bite prevention measures are equally important. If fever develops during travel or within 3 months of return, seek urgent medical attention and disclose your travel history. No malaria chemoprophylaxis is required for the Oklahoma leg.
Traveller-specific considerations (from health context)	Several factors specific to this traveller require particular attention. First, the combination of planned farm and animal contact in Kenya with no prior rabies vaccination represents a high-priority risk; the rabies pre-exposure prophylaxis series must be initiated as early as possible given the 21 to 28 day schedule required. Second, the traveller has no prior travel vaccines

documented and has not travelled internationally in the last 12 months, meaning this is effectively a high-risk first-time international trip requiring a comprehensive vaccine catch-up. Third, the traveller has indicated substance use adherence risk; this may affect medication adherence for malaria prophylaxis and other preventive measures. Discuss adherence strategies with your clinician, including pill reminders and the importance of completing the full prophylaxis course. Fourth, the traveller is travelling alone without a companion, which increases personal vulnerability in medical emergencies; ensure your travel insurance details and emergency contacts are stored on your phone and shared with someone at home. Fifth, the traveller has indicated volunteering and humanitarian activities; clarify the specific nature of these activities with your clinician as they may trigger additional bloodborne pathogen precautions and hepatitis B verification requirements.

MEDICATION LOGISTICS

PACKAGING	Carry all medications in original manufacturer packaging with pharmacy labels intact. Carry a physician letter listing all prescribed medications by both brand and generic name, particularly for any controlled substances or injectables.
SUPPLY RULE	Carry a minimum of double the required supply, split between carry-on and checked luggage, to protect against loss or delay.
DESTINATION LEGALITY VERIFIED	Yes — confirm all medications are legal at destination.

CLINICAL DECISION FLAGS

- TREE_3_MEDIUM_RISK_DESTINATION
- TREE_4_DURATION
- TREE_5_FLIGHT
- TREE_9_ANIMAL_FARM_CONTACT_RABIES_RISK
- TREE_9_VOLUNTEERING_HUMANITARIAN_BLOODBORNE_RISK
- TREE_14_CASUAL_SEXUAL_RELATIONSHIPS_INCONSISTENT_PROTECTION
- TREE_14_STI_HISTORY
- TREE_14_ALCOHOL_SUBSTANCE_USE_ADHERENCE_RISK
- TREE_12_NO_PRIOR_TRAVEL_VACCINES_DOCUMENTED
- TREE_13_NO_INTERNATIONAL_TRAVEL_LAST_12_MONTHS

SPECIALIST REFERRALS

CONDITION	SPECIALIST	URGENCY
Rabies pre-exposure prophylaxis required before departure given farm and animal contact activities in Kenya	Travel medicine physician or certified travel health clinic	BEFORE_TRAVEL

Comprehensive pre-travel vaccine catch-up required including yellow fever, hepatitis A, hepatitis B, typhoid, and rabies series	Travel medicine physician or certified travel health clinic	BEFORE_TRAVEL
Sexual health risk assessment and PrEP discussion given anticipated casual relationships, inconsistent condom use, and prior STI history in a high HIV-prevalence destination	Primary care physician or sexual health clinic	BEFORE_TRAVEL
Malaria chemoprophylaxis prescription required for Kenya leg	Primary care physician or travel medicine physician	BEFORE_TRAVEL

SEXUAL HEALTH & RISK BEHAVIOURS

Risk Level: HIGH

PREVENTION ADVICE

- Use condoms (male or female) consistently and correctly with every sexual encounter, including oral sex where appropriate.
- Carry an adequate supply of condoms from home; do not rely on availability or quality at the destination.
- Discuss HIV pre-exposure prophylaxis (PrEP) with your clinician before departure. PrEP is a daily oral medication that significantly reduces the risk of acquiring HIV and is appropriate given the anticipated risk profile.
- Know where to access HIV post-exposure prophylaxis (PEP) at your destination in Kenya and in Oklahoma. PEP must be started within 72 hours of a potential HIV exposure to be effective. In Nairobi, PEP is available at major hospitals and HIV clinics. In Oklahoma, PEP is available through emergency departments and sexual health clinics.
- Ensure hepatitis B vaccination is completed or initiated before departure.
- Avoid any procedures involving needles, tattooing, or piercing unless you can verify sterile professional facilities.
- Plan for post-travel STI screening and HIV testing after return, given prior STI history and anticipated risk behaviours.
- Alcohol consumption can impair judgment and increase the likelihood of unprotected sex; be mindful of this interaction.

PREP/PEP

PrEP/PEP availability discussed for high-risk destination.

AFTER YOU RETURN

RED FLAGS — SEEK IMMEDIATE CARE IF YOU EXPERIENCE

URGENT: Any fever at any point within 3 months of return from Kenya must be assessed as potential malaria immediately. Do not wait. Go to an emergency department or urgent care facility and state that you have recently travelled to Kenya. Malaria can be fatal if treatment is delayed.

WITHIN 1 WEEK

- Monitor for fever, chills, or flu-like symptoms. Any fever within 3 months of return from Kenya must be treated as a potential malaria emergency until proven otherwise. Seek urgent medical attention and inform the

clinician of your travel history.

- Monitor for gastrointestinal symptoms including diarrhoea, nausea, or abdominal pain, which may indicate a food or water-borne infection acquired in Kenya.
- If you had any potential HIV or STI exposure during travel, contact a sexual health clinic or your primary care physician within 72 hours for PEP assessment if applicable.

WITHIN 4 WEEKS

- Attend a post-travel health check with your primary care physician, particularly if you undertook volunteering, humanitarian, or animal contact activities.
- Report any skin rashes, sores, unusual discharge, or other symptoms that may indicate an STI acquired during travel.
- Complete the full course of malaria prophylaxis (atovaquone-proguanil must be continued for 7 days after leaving the malaria-risk area in Kenya).
- If you had any animal bite, scratch, or mucous membrane exposure in Kenya, seek immediate medical attention regardless of whether you received pre-exposure rabies prophylaxis. Post-exposure treatment is still required.

BEYOND 4 WEEKS

- Attend post-travel STI screening and HIV testing, particularly given the anticipated risk behaviours and prior STI history.
- If you experience persistent fatigue, unexplained weight loss, night sweats, or prolonged diarrhoea beyond 4 weeks after return, seek medical evaluation and disclose your travel history.
- Ensure any hepatitis B vaccination series initiated before travel is completed on schedule.
- If you received the rabies pre-exposure series, ensure the 12-month booster (if applicable to your schedule) is planned.

MEDICAL CARE & CONTACTS

CLINICS & FACILITIES

NAME	LOCATION & NOTES
Nairobi Hospital	Argwings Kodhek Road, Nairobi, Kenya +254 20 284 5000 · Central Nairobi Major private hospital with 24-hour emergency services, travel medicine, and HIV/PEP services.
Aga Khan University Hospital Nairobi	3rd Parklands Avenue, Nairobi, Kenya +254 20 366 2000 · Parklands, Nairobi International-standard private hospital with emergency department, infectious disease, and travel health services.
OU Health University of Oklahoma Medical Center	700 NE 13th Street, Oklahoma City, Oklahoma, United States +1 405 271 4700 · Oklahoma City Major academic medical centre with emergency and infectious disease services.

EMBASSY & CONSULAR

NAME	DETAILS
Nigerian High Commission, Nairobi, Kenya	Lenana Road, Hurlingham, Nairobi, Kenya. Tel: +254 20 271 4899. For consular assistance in Kenya.
Nigerian Embassy, Washington DC (for US matters)	3519 International Court NW, Washington DC 20008, United States. Tel: +1 202 986 8400.
US Embassy, Nairobi, Kenya	United Nations Avenue, Gigiri, Nairobi, Kenya. Tel: +254 20 363 6000. For emergency American Citizen Services.

EMERGENCY NUMBERS

LABEL	CONTACT
Kenya Emergency Services	999 or 112
Kenya Police	999
Nairobi Ambulance (AMREF Flying Doctors)	+254 20 699 2000
United States Emergency Services (Oklahoma)	911
International SOS (if covered by travel insurance)	Check your insurance policy for your dedicated assistance number

NEXT STEPS

- Book a pre-travel health consultation with a certified travel medicine clinic or your primary care physician immediately. Your departure date of 5 December 2026 means time is limited, particularly for the rabies pre-exposure series (21 to 28 days) and hepatitis B accelerated series.
- Initiate the rabies pre-exposure prophylaxis series as the highest priority given planned farm and animal contact in Kenya.
- Obtain yellow fever vaccination at a certified yellow fever vaccination centre and secure your International Certificate of Vaccination (yellow card) at least 10 days before departure.
- Discuss and obtain a prescription for malaria chemoprophylaxis (atovaquone-proguanil preferred) from your clinician.
- Discuss HIV PrEP with your clinician or sexual health clinic before departure given anticipated risk behaviours.
- Verify your travel insurance policy covers medical evacuation, pre-existing conditions, and healthcare costs in both Kenya and the United States.
- Confirm current US entry requirements for travellers arriving from Kenya, including any COVID-19 documentation or yellow fever certificate requirements.
- Store emergency contacts, your insurance assistance number, and the addresses of hospitals in Nairobi and Oklahoma on your phone before departure.
- Inform a trusted contact at home of your full itinerary, accommodation details, and expected check-in schedule.

This advisory has been generated by the TMAG (Travel Medicine Advisory Global) AI Clinical Decision Support System and is intended to inform a pre-travel health consultation with a qualified clinician. It does not constitute medical advice and must not replace a consultation with a licensed physician, travel medicine specialist, or pharmacist. All vaccine and medication recommendations require clinician review and a valid prescription where applicable. Information is based on CDC and WHO travel health guidelines current at the time of generation; destination risk profiles may change. Always check for the latest alerts before departure. Travellers with complex medical histories, chronic conditions, or pregnancy are strongly advised to seek specialist travel medicine review in person before travelling. Medication guidance in this report is advisory only and must not be used to alter, substitute, or discontinue any medication currently prescribed to you. Please consult your physician or pharmacist before acting on any medication recommendation. Copyright 2026 Travel Medicine Advisory Global. travelmedicine.global

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DOCTOR VERIFICATION & DIGITAL SIGNATURE

This travel health plan has been reviewed and approved by a licensed physician on the TMAG Doctor Network.

PHYSICIAN NAME Mat Smith

LICENSE NUMBER MT898320084

VALIDATION DATE 24 April 2026

STATUS **TMAG Verified & Approved**

Digital Signature



Official Stamp

TMAG Verified Seal

This document was generated by Travel Medicine Advisory Global and approved by a licensed physician.